TRAMILLE Capsule 50mg

NAME AND STRENGTH OF ACTIVE SUBSTANCE
Each capsule contains:
Tramadol Hydrochloride 50 mg

PRODUCT DESCRIPTION
White granule, capsule no.2, cap colored pink opaque and body colored maroon opaque.

INDICATIONS
Treatment of moderate to severe pain.

PHARMACODYNAMICS
Tramadol is a centrally acting synthetic analgesic compound. It is a non-selective pure agonist at mu, delta and kappa opioid receptors with a higher affinity for the mu receptor. Other mechanisms which may contribute to its analgesic effect are inhibition of neuronal reuptake of noradrenaline (norepineph. rine) and enhancement of serotonin release.

Tramadol opioid activity derives from low affinity binding of the parent compound to mu-opioid receptors and higher affinity binding of the active metabolite, O- desmethyl tramadol. Compared to morphine, tramadol does not show respiratory depression when given within the analgesic dosage interval. The effect of tramadol is considered 1/10 to 1/6 the effect of morphine. The gastrointestinal motility is not affected. There is minimal effect on the cardiovascular system. The contribution to human analgesia of tramadol relative to the active metabolite is unknown.

Tramadol has an antitussive effect. Animal studies have revealed a reduced dependence potential compared with morphine and a very slight tolerance potential.

PHARMACOKINETICS

Absorption
Racemic tramadol is rapidly and almost completely absorbed after oral administration. The mean absolute bioavailability of a 100 mg oral dose is approximately 75%. The mean peak plasma concentration of racemic tramadol and M1 occurs at 2 and 3 hours, respectively, after administration in healthy adults.

Distribution
The volume of distribution of tramadol was 2.6 and 2.9 liters/kg in male and female subjects, respectively, following a 100 mg intravenous dose. The binding of tramadol to human plasma proteins is approximately 20%.

Metabolism
Tramadol is extensively metabolized by a number of pathways, including CYP2D6 and CYP3A4, as well as by conjugation of parent and metabolites. Approximately 30% of the dose is excreted in the urine unchanged, 60% as metabolites and the remainder is excreted either as unidentified or as unextractable metabolites. The major metabolic pathways appear to be N- and O-demethylation and glucuronidation or sulfation in the liver.

Elimination
Tramadol and its metabolites are eliminated primarily through the kidneys. The mean terminal plasma elimination half-lives of racemic tramadol and racemic M1 are 6.3 ± 1.4 and 7.4 ± 1.4 hours, respectively. The plasma elimination half-life of racemic tramadol increased from approximately 6 to 7 hours upon multiple dosing.

RECOMMENDED DOSAGE
Oral dosage should be adjusted according to the intensity of pain.

Adults and adolescents over 12 years of age:
Tramille Capsule 50mg is not approved for use in patients below 12 years old.

Pediatric population:
The safety and efficacy of Tramille Capsule 50mg has not been studied in pediatric population. Therefore, use of Tramille Capsule 50mg is not recommended in patients under 12 years of age.

In general the daily dose:
1 to 2 capsules to be taken every 4 to 6 hours as needed, up to a maximum of 8 capsules daily.

Dose Adjustment
Patients with liver and renal impairment: Creatinine clearance < 30 mL/minute: 50 – 100 mg every 12 hours, maximum 200 mg a day.
Patients with liver cirrhosis: 50 mg every 12 hours.

DOSAGE AND ADMINISTRATIONS
Oral

CONTRAINDICATIONS
Children under 12 years of age. The safety and efficacy of Tramille has not been studied in the paediatric population.

Children younger than 18 years old, to treat pain after surgery to remove the tonsils and/or adenoids.

Adolescents between 12 and 18 years old, who are obese or have conditions such as obstructive sleep apnea or severe lung disease, which may increase the risk of serious respiratory depression.

Patients with hypersensitivity to tramadol or opioid in general.

Patients on MAOI treatment within the last 14 days.

Acute intoxication with alcohol, hypnotics, analgesic or other CNS-actin drugs.

WARNINGS AND PRECAUTIONS
As an opioid analgesic, tramadol has a tendency of causing abuse and misuse. Use with particular caution in opioid-dependent patients.

Use with caution in head injury, increasing intracranial pressure, severe liver and kidney insufficiency because it can increase the risk of convulsion or shock.

Care should be taken when treating patients with respiratory depression, or if concomitant CNS depressant drugs are being administered, or if the recommended dosage is significantly exceeded, as the possibility of respiratory depression cannot be excluded in these situations.

Driving and Operating Machineries

Tramadol has significant effect on mental alertness that patients must not drive a vehicle or operate dangerous machinery while using it.

Paediatric Population
The safety and efficacy of Tramille has not been studied in the paediatric population. Therefore, use of Tramille is not recommended in patients under 12 years of age.

Respiratory depression
Administer Tramille cautiously in patients at risk for respiratory depression, including patients with substantially decreased respiratory reserve, hypoxia, hypercapnia, or pre-existing respiratory depression, as in these patients, even therapeutic doses of Tramille may decrease respiratory drive to the point of apnea. In these patients, alternative non-opioid analgesics should be considered. When large doses of tramadol are administered with anaesthetic medications or alcohol, respiratory depression may result. Respiratory depression should be treated as an overdose. If naloxone is to be administered, use cautiously because it may precipitate seizures.

Cytochrome P450 (CYP)2D6 Ultra-rapid Metabolism

Some individuals may be CYP2D6 ultra-rapid metabolisers. These individuals convert tramadol more rapidly than other people into its more potent opioid metabolites O-desmethyltramadol (M1). This rapid conversion could result in higher than expected opioid-like side-effects including life-threatening respiratory depression. The prevalence of this CYP2D6 phenotype varies widely and has been estimated at 0.5 to 1% in Chinese, Japanese and Hispanics, 1 to 10% in Caucasians, 3% in African Americans, and 16 to 28% in North African, Ethiopians, and Arabs. Data are not available for other ethnic groups.

INTERACTION WITH OTHER MEDICAMENTS

In patients treated with MAO inhibitors in the 14 days prior to the use of the opioid, life-threating interactions on the central nervous system, respiratory and cardiovascular function have been observed.

"Concomitant administration of Tramadol with CNS depressants, including alcohol, may cause profound sedation, respiratory depression, coma and death."

Simultaneous or previous administration of carbamazepine (enzyme inducer) may reduce the analgesic effect and duration of action.

Tramadol can induce convulsions and increase the potential for Selective Serotonin Reuptake Inhibitors (SSRIs), Selective Noradreanaline Reuptake Inhibitors (SNRIs), tricyclic antidepressants, antipsychotics and other seizure threshold-lowering drugs (e.g. bupropion, mirtazapine) to cause convulsions.

Concomitant treatment with tramadol and coumarin derivatives (e.g. warfarin) has been reported of increased INR with major bleeding in some patients.

The same interaction with MAO inhibitors cannot be ruled out during treatment with tramadol.

PREGNANCY AND LACTATION

Pregnancy

Tramadol has been shown to cross the placenta. There are no adequate and well-controlled studies in pregnant women. Safe use in pregnancy has not been established. Tramille is not recommended for pregnant women.

Lactation

Approximately 0.1% of the maternal dose of tramadol is excreted in breast milk. In the immediate post-partum period, for maternal oral daily dosage up to 400 mg this corresponds to a mean amount of tramadol ingested by breast-fed infants of 3% of the maternal weight-adjusted dosage. For this reason tramadol should not be used during lactation treatment with tramadol. Discontinuation of breastfeeding is generally not necessary following a single dose of tramadol.

SIDE EFFECTS

The common side effects are dizziness, sedation, fatigue, headache, pruritus, sweating, skin rashes, dry mouth, nausea, vomiting, dyspepsia and constipation. Respiratory depression (rare).

SYMPTOMS AND TREATMENT OF OVERDOSE

Symptoms

In principle, on intoxication with tramadol symptoms similar to those of other centrally acting analgesics (opioids) are to be expected. These include in particular miosis, vomiting, cardiovascular collapse, consciousness disorders up to coma, convulsions and respiratory depression up to respiratory arrest.

Treatment

The general emergency measures apply. Keep open the respiratory tract (aspiration), maintain respiration and circulation depending on the symptoms. The antidote for respiratory depression is naloxone. In animal experiments naloxone had no effect on convulsions. In such cases diazepam should be given intravenously.

In case of intoxication orally, gastrointestinal decontamination with activated charcoal or by gastric lavage is only recommended within 2 hours after tramadol intake. Gastrointestinal decontamination at a later time point may be useful in case of intoxication with exceptionally large quantities or prolonged-release formulation.

DOSAGE FORM AND PACKAGING

Capsule - Alu/PVC blister of 10 x 10's/box and 50 x 10's/box.

STORAGE

Store at room temperature (below 30°C), in the original package. Keep out of reach of children.

SHELF LIFE

3 years

PRODUCT REGISTRATION HOLDER

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